HEALTH BEACON

CMSRN EXAM PREP

Practice Exam 01 - Professional Concepts

19 scored practice questions | Printable Practice Exam

Instructions	Select the single best answer for each question. An answer key with explanations begins after the question section.

PART A - QUESTIONS

1.

Scenario

A patient with limited English proficiency is being prepared for a nonemergent procedure. The patient nods during the explanation, but when asked to describe the procedure, gives an unrelated answer. The patient's adult daughter offers to interpret.

Which action by the nurse best supports accurate, patient-centered communication?

A) Ask the daughter to translate because she knows the patient's usual expressions

B) Obtain a qualified medical interpreter and speak directly to the patient

C) Give the patient the written English consent form and allow additional reading time

D) Ask only yes-or-no questions to reduce the chance of misunderstanding

2.

Scenario

At change of shift, a medical-surgical nurse receives reports on four patients. One patient awaiting discharge needs a prescription clarified. One reports incisional pain of 7 on a 0-to-10 scale 45 minutes after an analgesic. One has new restlessness, a blood pressure decrease from 128/74 to 92/58 mm Hg, and urine output of 20 mL in the past hour. One needs teaching before a scheduled afternoon test.

Which patient should the nurse assess without delay?

A) The patient awaiting prescription clarification

B) The patient with persistent incisional pain

C) The patient needing pretest teaching

D) The patient with restlessness, hypotension, and low urine output

3.

Scenario

Two nurses are preparing to transfer a bariatric patient from bed to chair. The ceiling lift is being used elsewhere, and one nurse suggests completing a manual transfer because the unit is busy. The patient cannot reliably bear weight.

Which response best promotes a safe practice environment?

A) Complete the transfer with a gait belt and request that the patient push with both arms

B) Ask two additional staff members to help with a manual transfer

C) Delay the transfer until appropriate lift equipment and trained assistance are available

D) Transfer the patient only if both nurses agree they can manage the patient's weight

4.

Scenario

A competent adult with gastrointestinal bleeding refuses a blood transfusion after the provider explains the risks, benefits, and alternatives. The patient's spouse tells the nurse, "Ignore that decision and give the blood. I am the spouse."

Which nursing action is appropriate?

A) Ask the spouse to sign the transfusion consent as next of kin

B) Confirm that the refusal is informed, notify the provider, document it, and honor the patient's decision

C) Delay documenting the refusal until the patient's hemoglobin is rechecked

D) Request an emergency court order because refusal may lead to serious harm

5.

Scenario

A medical-surgical unit's fall rate has increased for three consecutive months. Staff members propose purchasing new alarms immediately, but the nurse leading the improvement work has not yet reviewed individual fall events.

Which action provides the strongest starting point for improvement?

A) Analyze fall-event data for patterns in timing, location, patient factors, and contributing processes

B) Purchase the alarms because delaying an intervention could allow another fall

C) Reeducate all staff on the current fall policy and remeasure the rate next quarter

D) Compare individual nurses' fall counts and counsel those with the highest numbers

6.

Scenario

A patient admitted with pneumonia has an early-warning score that has risen from 3 to 7 in two hours. Current findings include respiratory rate 32/min, oxygen saturation 88% on 4 L/min by nasal cannula, new confusion, and heart rate 124/min. The nurse has repositioned the patient and verified the oximeter reading.

Which next step demonstrates appropriate use of deterioration resources?

A) Recalculate the score in 30 minutes to determine whether the trend continues

B) Send a routine message to the provider and continue the scheduled medication pass

C) Activate the rapid response process while continuing immediate supportive assessment and care

D) Transfer the patient to a chair to improve ventilation while waiting for new orders

7.

Scenario

A patient with hearing loss is preparing for discharge with a new anticoagulant. The patient has hearing aids but is not wearing them and says, "Just give the instructions to my neighbor."

Which approach best verifies that the patient understands the medication plan?

A) Ask the neighbor to repeat the instructions to the patient at home

B) Speak loudly from the doorway and provide the standard printed handout

C) Ask the patient to sign that the instructions were received

D) Reduce background noise, support use of the hearing aids, face the patient, and use teach-back

8.

Scenario

Barcode scanning alerts a nurse that a rapid-acting insulin syringe prepared for one patient was selected for a different patient. The insulin is not administered, and the correct medication is given.

How should the nurse respond to this near miss?

A) Document the error in the patient's progress note because the wrong syringe reached the bedside

B) Submit a safety report with objective details so system contributors can be evaluated

C) Take no further action because the scanning system prevented patient harm

D) Report the event only if the same selection error occurs again during the shift

9.

Scenario

A nurse reports ongoing distress about providing burdensome treatments that appear inconsistent with a patient's previously stated goals. The nurse says, "I know what should be discussed, but I feel powerless and dread coming to work."

Which response by the charge nurse is most constructive?

A) Remind the nurse that treatment decisions belong exclusively to the provider

B) Reassign the patient permanently so the nurse no longer encounters the conflict

C) Encourage the nurse to separate personal values from work and avoid discussing the case

D) Acknowledge the moral distress, review available escalation pathways, and facilitate an ethics consultation or debriefing

10.

Scenario

A unit council is considering changing its practice for preventing moisture-associated skin damage. A member presents one small observational study and recommends adopting its intervention throughout the hospital immediately.

Which response best reflects evidence-based practice?

A) Adopt the intervention because any published study is stronger than local practice

B) Reject the study because observational evidence cannot contribute to practice decisions

C) Develop a focused clinical question, appraise the body of evidence, and integrate it with expertise and patient factors

D) Wait until a national regulatory agency mandates one specific intervention

11.

Scenario

A nurse is preparing discharge documents while an unlicensed assistive person obtains routine vital signs. Another patient uses the call light and reports new pressure in the center of the chest. A scheduled admission is expected in 10 minutes.

How should the nurse reorganize the work?

A) Finish the discharge because transportation is waiting, then evaluate the chest pressure

B) Assess the patient with chest pressure now and redistribute routine tasks that can safely be delegated

C) Ask the assistive person to characterize the chest pressure while the nurse prepares for the admission

D) Call the receiving unit to delay the admission before gathering more information about the chest pressure

12.

Scenario

An experienced licensed practical/vocational nurse (LPN/LVN) is available to help an RN who has just received a patient from the emergency department with possible sepsis and unstable vital signs. Unit policy and the applicable nurse practice act reserve initial comprehensive assessment and care planning for the RN.

Which assignment is appropriate?

A) The RN performs the initial comprehensive assessment and delegates selected predictable tasks after determining the plan

B) The LPN/LVN performs the initial assessment, and the RN signs it after reviewing the findings

C) The LPN/LVN establishes the nursing diagnoses while the RN contacts the provider

D) The RN delegates all admission activities because the LPN/LVN is experienced

13.

Scenario

A hospitalized patient enrolled in a research study tells the bedside nurse, "I want to stop, but the researcher implied my doctors might be disappointed and my care could change."

Which response by the nurse best protects the patient's rights?

A) Advise the patient to remain enrolled until discharge to avoid incomplete data

B) Ask the attending provider whether withdrawal would affect the treatment plan

C) Explain that consent cannot be withdrawn after study procedures have begun

D) State that participation is voluntary and care should not be penalized, then contact the research team through the appropriate process

14.

Scenario

Two medical-surgical units are comparing hospital-acquired pressure injury data. Unit A had 4 injuries during 2,000 patient-days; Unit B had 5 injuries during 4,500 patient-days. A manager states that Unit B performed worse because it had more injuries.

What is the quality nurse's best response?

A) Compare standardized rates using a common denominator and examine differences in patient risk and processes

B) Accept the conclusion because the unit with more events always has poorer performance

C) Exclude Unit B's data because units of different sizes cannot be compared

D) Compare only monthly census totals because patient-days distort infrequent outcomes

15.

Scenario

A nurse calls a provider about a patient with increasing abdominal distention, tachycardia, and decreasing blood pressure after a procedure. The provider responds, "Some distention is expected," and attempts to end the call. The nurse remains concerned about deterioration.

Which communication strategy is most appropriate?

A) Document that the provider was notified and wait for the next scheduled assessment

B) State the concern and safety risk clearly, request a specific evaluation, and use the chain of command if the concern remains unresolved

C) Tell the provider that any adverse outcome will be the provider's responsibility

D) Ask another nurse to call with the same information without mentioning the first conversation

16.

Scenario

Over several weeks, a unit manager observes frequent missed breaks, rising sick calls, emotional exhaustion, and a decline in staff willingness to help one another. Telling staff to practice self-care has not improved the pattern.

Which intervention is most likely to address the work-system contributors to burnout?

A) Provide a voluntary mindfulness link and keep staffing processes unchanged

B) Ask resilient staff members to cover additional shifts until morale improves

C) Review workload and staffing demands, protect recovery time, and connect staff with confidential support resources

D) Require staff to attend a session on maintaining a positive attitude after work

17.

Scenario

A patient's scheduled pain medication was delayed for 90 minutes because an electronic request was routed to the wrong queue. After receiving the medication, the patient says, "No one cared that I was suffering."

Which nursing response best demonstrates service recovery and supports improvement?

A) Explain that electronic routing is controlled by another department

B) Reassure the patient that delays are common during high census periods

C) Ask the patient to discuss the concern on the satisfaction survey after discharge

D) Acknowledge the experience, apologize, address current needs, explain follow-up, and report the process problem

18.

Scenario

During a busy medication pass, a nurse enters the room of a patient with a chest tube and finds the patient newly short of breath. The drainage system is upright, and continuous bubbling is visible in the water-seal chamber. A colleague says, "Just clamp the tube so you can finish medications."

Which action best demonstrates sound problem solving?

A) Stay with the patient, assess respiratory status and the system for a disconnection or leak, and obtain urgent help as indicated

B) Clamp the tube near the patient's chest until the medication pass is complete

C) Increase wall suction until bubbling stops, then reassess after medications

D) Record the bubbling as an expected finding because the drainage system is upright

19.

Scenario

After caring for a patient with an unusual injury, a nurse wants to post a clinical lesson in a private professional social-media group. The proposed post omits the patient's name but includes the age, uncommon injury, admission date, and hospital location.

Which action is ethically and professionally appropriate?

A) Post the information because a private professional group is not considered public

B) Do not post the case details; use approved education channels that protect confidentiality

C) Post after asking coworkers to confirm that the patient's name is not included

D) Post after discharge because confidentiality obligations end when care is completed

PART B - ANSWER KEY AND EXPLANATIONS

Question numbering below corresponds to the scored questions in Part A.

1	B	2	D	3	C	4	B	5	A
6	C	7	D	8	B	9	D	10	C
11	B	12	A	13	D	14	A	15	B
16	C	17	D	18	A	19	B		

1. Correct: B - Obtain a qualified medical interpreter and speak directly to the patient

A qualified interpreter supports accurate two-way communication and preserves the patient's role in decisions. Relying on family, English-only materials, or yes-or-no questions does not adequately verify understanding.

2. Correct: D - The patient with restlessness, hypotension, and low urine output

This cluster suggests impaired perfusion and possible acute deterioration, requiring immediate assessment. The other needs are important but do not present the same time-sensitive physiologic risk.

3. Correct: C - Delay the transfer until appropriate lift equipment and trained assistance are available

A patient who cannot reliably bear weight requires appropriate patient-handling equipment. Adding staff or relying on judgment about weight does not make an unsafe manual lift acceptable.

4. Correct: B - Confirm that the refusal is informed, notify the provider, document it, and honor the patient's decision

A capable adult may refuse treatment even when refusal carries serious risk. The spouse cannot override the patient's informed choice, and the nurse should communicate and document it promptly.

5. Correct: A - Analyze fall-event data for patterns in timing, location, patient factors, and contributing processes

Reviewing process and outcome data helps identify root contributors and select targeted interventions. Purchasing equipment or broadly reeducating staff before analysis may not address the actual causes.

6. Correct: C - Activate the rapid response process while continuing immediate supportive assessment and care

The worsening score plus hypoxemia, tachypnea, tachycardia, and confusion indicates acute deterioration. Rapid-response activation mobilizes timely expertise while bedside assessment and support continue.

7. Correct: D - Reduce background noise, support use of the hearing aids, face the patient, and use teach-back

Removing communication barriers and asking the patient to explain the plan in their own words directly verifies understanding. A signature or handout alone confirms neither access nor comprehension.

8. Correct: B - Submit a safety report with objective details so system contributors can be evaluated

Near misses reveal hazards before harm occurs and should be reported through the safety system. The report supports learning and process redesign; it is not a substitute for clinical charting.

9. Correct: D - Acknowledge the moral distress, review available escalation pathways, and facilitate an ethics consultation or debriefing

Recognizing the conflict and connecting the nurse and team with ethics and debriefing resources addresses both the unresolved care concern and its effect on staff. Avoidance or dismissal can intensify moral distress.

10. Correct: C - Develop a focused clinical question, appraise the body of evidence, and integrate it with expertise and patient factors

Evidence-based practice integrates critically appraised evidence with clinical expertise and patient circumstances or preferences. One study alone should neither trigger automatic adoption nor be dismissed solely because of its design.

11. Correct: B - Assess the patient with chest pressure now and redistribute routine tasks that can safely be delegated

New chest pressure requires prompt RN assessment. Reprioritizing and safely delegating routine work preserves attention for the unexpected high-risk change.

12. Correct: A - The RN performs the initial comprehensive assessment and delegates selected predictable tasks after determining the plan

The RN retains responsibilities that policy and the nurse practice act reserve for RN judgment, including the initial comprehensive assessment of an unstable patient. Experience does not expand another team member's legal scope.

13. Correct: D - State that participation is voluntary and care should not be penalized, then contact the research team through the appropriate process

Research participation must remain voluntary, and participants may withdraw without coercive loss of clinical care. The nurse should support the patient's expressed choice and activate the study's process for addressing consent concerns.

14. Correct: A - Compare standardized rates using a common denominator and examine differences in patient risk and processes

Raw event counts do not account for different exposure volumes. Standardized rates and relevant risk or process information make the comparison more meaningful.

15. Correct: B - State the concern and safety risk clearly, request a specific evaluation, and use the chain of command if the concern remains unresolved

Graded assertiveness, a clear request, and escalation through the chain of command support timely action when deterioration concerns are dismissed. Documentation alone does not protect the patient from an unresolved risk.

16. Correct: C - Review workload and staffing demands, protect recovery time, and connect staff with confidential support resources

Burnout prevention requires attention to work-system demands as well as access to individual support. Asking depleted staff to do more or relying only on attitude and self-care interventions leaves the drivers unchanged.

17. Correct: D - Acknowledge the experience, apologize, address current needs, explain follow-up, and report the process problem

Effective service recovery validates the patient's experience, responds to current needs, communicates follow-up, and feeds the system issue into improvement work. Deflection and delayed feedback do not repair trust.

18. Correct: A - Stay with the patient, assess respiratory status and the system for a disconnection or leak, and obtain urgent help as indicated

New dyspnea with continuous water-seal bubbling requires immediate patient assessment and investigation for an air leak or disconnection. Routine clamping can create dangerous pressure and should not be used to postpone evaluation.

19. Correct: B - Do not post the case details; use approved education channels that protect confidentiality

Multiple indirect identifiers can make a patient recognizable even without a name. Confidentiality continues after discharge, and professional education should use approved, privacy-protective processes.